جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايمان محمد محمد رمضان
المحافظة : الجيزه **الحي / المدينة :** العمرانيه **العنوان :** 0
رقم البطاقة : 27903120103964 **رقم التليفون :** 01006501720 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 12-03-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايمان محمد محمد رمضان
National ID: 27903120103964

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٠١١٠٧٣١٣٢٧٨
٠١٠٠٦٥٠١٤٢٠



National ID - Front & Back
[[DOCTYPE ID_FRONT_BACK]]

GUSTAVE ROUSSY

جوستاف روسي

atient Name	ايمان محمد محمد رمضان 0	Code Number	YK 275939
ex/Age	Female 47Y	Patient ID	20628
egistered Date	11/06/2026 08:38:56	Referred By Dr.	Out Patient
alidation Date	04/07/2026 14:11:51	Branch	Gustave Roussy
eporting Date	7/4/2026, 4:23:56 PM		

breast slide revision

Breast Slide Revision

Clinical Data: Right breast 9 O'clock mass, BIRADs 5.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:
- One referred block labelled by (1508-26). Together with, four referred immunostained slides for (ER, PR, Her2 & SISH).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* Necrosis: Focal.

IPT:
- ER: Moderate nuclear reaction in 90% of tumor cells (score 7/8).
- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified with wide small clusters.

Diagnosis:
Right breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 3.

IPT:
- ER: Positive (7/8).
- PR: Positive (5/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with wide small clusters.

immunohistochemistry
[[DOCTYPE RIC]]

Name: ايمان محمد محمد رمضان **MR. :** 11435	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 47-year-old pre-menopausal individual with a negative family history and no documented co-morbidities. The clinical history includes a diagnosis of invasive ductal carcinoma, grade 3.

Core biopsy demonstrated invasive ductal carcinoma, grade 3, with estrogen receptor positivity 7/8, progesterone receptor positivity 5/8, and HER2 score +2 with amplification. Imaging revealed an ACR B sonomammographic pattern. On the right side, a 9-10 o'clock upper outer quadrant mass measuring 5 × 2 cm was noted, extending to the nipple, with retroareolar nodules causing nipple retraction on examination and a single level I pathological lymph node. The left breast showed mild fibrocystic changes corresponding to BIRADS 2. Staging workup with CT of the chest, abdomen, and pelvis and a bone scan were both negative for metastatic disease.

The referring team recommended clips placement, neoadjuvant chemotherapy (NACT), followed by radiotherapy and central quadrantectomy with lymph node surgery according to response. The multidisciplinary team agreed on clips placement, insertion of a port-a-cath, administration of NACT with TCHP for six cycles, and subsequent radiotherapy with central quadrantectomy and lymph node surgery based on treatment response.

تحريرا في ,

التاريخ :	23-09-2026	الوقت :	

الطبيب المعالج :

يعتمد ,

atient Name	ايمان محمد محمد رمضان 0	Code Number	YK 275939
ex/Age	Female 47Y	Patient ID	20628
egistered Date	11/06/2026 08:38:56	Referred By Dr.	Out Patient
alidation Date	04/07/2026 14:11:51	Branch	Gustave Roussy
eporting Date	7/4/2026, 4:23:56 PM		

* *Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* **Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations.IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd El-Atty.
Dr. Walaa Ibrahim Ghonaim, MD.

Liver Function

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immunohistochemistry
[[DOCTYPE RHC]]

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

07/26/2026

MDT FORM

Name:	ايمان محمد د محمد رمضان	**MRN:**	11435
Age:	47 Y	**ID:**	27903120103964
Menopausal Sta	PRE	**Phone:***	01107313328
NRS SCORE:	PS 0	**Marital Status:**	MARRIED
BMI:	30	**Family History:**	-VE
Gender			
CO Morbidity:	NAD	**Reason Of MDT;**	NEW CASE
Surgical History:*	NORMAL DELIVARIES	**ECOG Performance Status**	◉ 0 ○ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC G3

ER: 7/8 **PR:** 5/8 **HER2:** +2 (AMP **KI67:**

History:

R L

TNM

Sonomammography:
ACRB
RT 9-10 CLOCK UOQ MASS 5 X 2CM AND REACHING NIPPLE
RT RETROAREOLAR NODULES WITH NIPPLE RETACTION BY EXAM
RT SINGLE LEVEL I PATHOLOGICAL LN
LT MILD FIBROCYSTIC CHANGES (BIRADS 2)

Biomarker/Molecular
BY EXAM MASSES AT ZONE A B

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Mets Workup:
CT CAP ,,,,, FREE.
BONE SCAN ,,,,,,, FREE.

Recommendations:
FOR CLIPS BRACKTING, NACT ,THEN RT CENTRAL QUADRENTECTOMY +LNS ACCORDING RESPONSE

MDT Outcome & Plan
FOR CLIPS BRACKTING
PORTACATH INSERTION
NACT: TCHP X 6 cycles
THEN RT CENTRAL QUADRENTECTOMY +LNS ACCORDING TO RESPONSE

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د خالد عبد العزيز	د محمد النيروي		

PATIENT NAME: EMAN MOHAMMED MOHAMMED RAMADAN **GENDER:** Female **DATE :** 21-May-2026 10:48 AM	**PATIENT ID:** 10126280558 **AGE:** 47 y **ORDER:** 3879846

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **21-May-2026 05:28 PM**

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مستشفى بهية الشيخ زايد
٤ الحي الأول - الشيخ زايد أمام كلية الهندسة

PATIENT NAME: EMAN MOHAMMED MOHAMMED RAMADAN **GENDER:** Female **DATE :** 03-Jun-2026 12:10 PM	**PATIENT ID:** 10126280558 **AGE:** 47 y **ORDER:** 3879837,3879892

Multislice post contrast CT scan of the chest, abdomen and pelvis revealed:

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 90 ml.

FINDINGS:

(I) Chest:

- The right breast shows a nodular enhancing lesions, the largest is seen measuring about 22 x 21 mm in its maximum axial dimensions, with irregular margins and surrounding mild parenchymal edematous changes.
- Enlarged right axillary lymph node is seen in level I measuring about 17 x 16 cm in its maximum axial dimensions, as well as few prominent ones in level I and II.
- Aberrant right subclavian artery is noted (variant).
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

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- Bulky uterus with anterior wall myometrial homogenous lesion measuring 25 mm likely Fibroid.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Small left ischial sclerotic lesion is seen, Bone island.
- Dorsolumbar spondylo-degenerative changes.

OPINION:

- **Right breast malignant-looking enhancing nodular lesion with metastatic-looking right axillary lymphadenopathy.**
- **No CT evidence of pulmonary, mediastinal, hilar, hepatic, adrenal, abdominal nodal, pelvic nodal, or peritoneal metastatic disease detected on the current study.**

Reported By,
Dr. Ahmed Abdel-Monem, MSc
Dr. Amr Farouk, MD **04-Jun-2026 05:51 AM**

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مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

PATIENT NAME: EMAN MOHAMMED MOHAMMED RAMADAN **GENDER:** Female **DATE :** 02-May-2026 12:27 PM	**PATIENT ID:** 10126280558 **AGE:** 47 y **ORDER:** 3850689,3850690,3853800

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND



INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Right UOQ irregular spiculated high density lesions all area measuring about 5x2cm .
- This was confirmed by Complementary Ultrasound which showed right 9-10 o'clock irregular spiculated interconnected hypoechoic solid mass lesions measuring about 1.5x0.7cm and 1.9x1.7cm with adistance1. 2cm in between the lesions being 0.5cm from the overlying skin and reaching the nipple
- Left retroareolar 12 o'clock hypoechoic nodules measuring 0.5x0.6 and 0.7x0.5cm with no detected internal vascularity by color doppler study .
- Bilateral scattered anechoic cysts .
- No left solid mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right single pathological level I axillary lymph node measuring 1.8x0. 6cm with cortical thickening 0.7cm .
- Left nonspecific axillary lymph nodes.

OPINION:

- Right 9-10 o'clock malignant looking mass lesions with ipsilateral single level I pathological lymph node as described for histopathological correlation (BIRADS 5)
- Right 12 o'clock in determinant nodules
- Left mild fibrocystic changes (BIRADS2).

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مستشفى بهية الشيخ زايد
٤ الحي الأول الشيخ زايد أمام كلية الهندسة





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مستشفى بهية الشيخ زايد
٤ الحي الاول الشيخ زايد امام كلية الهندسة

atient Name	ايمان محمد محمد رمضان 0	Code Number	YK 275939
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egistered Date	11/06/2026 08:38:56	Referred By Dr.	Out Patient
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eporting Date	7/4/2026, 4:23:56 PM		

breast slide revision

Breast Slide Revision

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Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* Necrosis: Focal.

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- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
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- SISH: Amplified with wide small clusters.

Diagnosis:
Right breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 3.

IPT:
- ER: Positive (7/8).
- PR: Positive (5/8).
- Her2: Equivocal (score 2+).
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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
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the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd El-Atty.
Dr. Walaa Ibrahim Ghonaim, MD.

Liver Function

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MRN:	10126280558	D.O.B:	12/03/1979
Patient Name:	EMAN MOHAMMED MOHAMMED RAMADAN		
IC No.:	27903120103964	Sex:	Female
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2605030209	**Collection Date**	03/05/2026 02:31 PM
Accession No	SUR-26-1508	**Received Date**	03/05/2026 02:31 PM
Sample Type	Tissue	**Doctor**	DALIA MAHMOUD MOHAMED MAHMOUD

Clinical Information

Right breast 9 o` clock lesion, BIRADs 5, US guided biopsy.

Gross Picture

Multiple tissue cores, ranging in size from 1 cm to 1.5 cm long, Totally submitted.

Microscopic Picture

A case of invasive duct carcinoma ,grade 3. Focal tumor necrosis is seen. No DCIS component nor lymphovascular emboli were detected .Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, grade 3

Laterality

Right

Specimen type:

U/S guided Tru-cut biopsy.

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

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٤ المحور الأول - الشيخ زايد أمام كلية الهندسة

Histological grade (1-3):
3

Diagnosis

Right breast 9 o`clock lesion, US guided biopsy:

Invasive duct carcinoma, grade 3.

Comments
ER, PR & Her2 are ordered.

Biopsy reporting category
B5b= Invasive cancer

Signature
Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Co-signed by:
Heba A Abdelbaky (MD)

Reported and electronically endorsed by
heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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MRN:	10126280558		
Patient Name:	EMAN MOHAMMED MOHAMMED RAMADAN	D.O.B:	12/03/1979
IC No.:	27903120103964	Sex:	Female
Location:	External Clinic		

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2605050176	**Collection Date**	05/05/2026 12:05 PM
Accession No	IMM-26-1705	**Received Date**	05/05/2026 12:05 PM
Sample Type	Paraffin Block	**Doctor**	Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
1508-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%–100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

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- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
Right

Diagnosis :
ER: Positive, weak nuclear reaction in 80% of tumor cells [6/8].
PR: Positive, intermediate nuclear reaction in 25% of tumor cells [5/8].

Signature
Heba A Abdelbaky (MD)

Co-Signed by:
Marwa Mahmoud Abd Elkader, FEBP , IFCAP.

Reported and electronically endorsed by
Marwa Mahmoud Abdelkader Nada

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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مستشفى بهية الشيخ زايد

MRN:	10126280558	D.O.B:	12/03/1979
Patient Name:	EMAN MOHAMMED MOHAMMED RAMADAN		
IC No.:	27903120103964	Sex:	Female
Location:	External Clinic		

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id	2605050177	**Collection Date**	05/05/2026 12:05 PM
Accession No	IMM-26-1706	**Received Date**	05/05/2026 12:05 PM
Sample Type	Paraffin Block	**Doctor**	Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
1508-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO–CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within <=10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within >10% of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in >10% of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in >10 % of tumor cells.

(Laterality) breast Cancer:
Right

Laterality :
Right

Her2 Result
Equivocal [score 2+]. Recommended For SISH

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4 شارع علوية أول شارع الهرم بجوار داري , الجيزة
مستشفى بهية الشيخ زايد
3 الحي الأول , الشيخ زايد أمام كلية الهندسة

Signature
Heba A Abdelbaky (MD)

Reported and electronically endorsed by
heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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4 شارع علوية أول شارع الهرم بجوار داري , الجيزة
مستشفى بهية الشيخ زايد

MRN:	10126280558		
Patient Name:	EMAN MOHAMMED MOHAMMED RAMADAN	D.O.B:	12/03/1979
IC No.:	27903120103964	Sex:	Female
Location:	External Clinic		

MOLECULAR GENETICS

Sish

Specimen Id	2605090260	**Collection Date**	09/05/2026 11:52 AM
Accession No	SISH-26-167	**Received Date**	09/05/2026 11:52 AM
Sample Type	Tissue	**Doctor**	Mohamed Mostafa Mohamed Mostafa

Sish Report

pathology number
1508/26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for HER 2 detection by silver in situ hybridization.

System Used :
Ventana BenchMark ULTRA autostainer.

Probe:
Ventana Inform HER2 Dual ISH DNA Probe Cocktail

Scoring system :
Reference: ASCO–CAP HER2 Test Guideline Recommendations,2018 Twenty nuclei (each containing red (Chr17) and black (HER2) signals) should be enumerated. The final results for the HER2 status are reported based on the ratio formed by dividing the sum of HER2 signals for all 20 nuclei divided by the sum of Chromosome 17 signals for all 20 nuclei. The amplification status is defined as Amplified if the HER2/Chromosome 17 ratio >/=2.0 and the average Her2 gene copy number is >/=4.0. It is Non-Amplified if the HER2/Chromosome 17 ratio <2.0 with the Her2 gene copy number is <4.0.
If the HER2/Chr17 ratio is <2 and the Her2 gene copy number is between 4.0 and 6.0, or, HER2/Chr17 ratio is >/=2 and the Her2 gene copy number is <4, or HER2/Chr17 ratio is <2 and the Her2 gene copy number is >/= 6.0, an additional work should be done.

(Laterality) breast Cancer:
Right

Site 1

Laterality #1

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Right

Diagnosis #1

Rt. breast Cancer:
Her-2 gene: Amplified (Her2/Chr17 =2.04 and average Her2 copy number is 5.1).

Final Result #1

Amplified

Site 2

Signature

Heba A Abdelbaky (MD)

Co-Signed by:

Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Reported and electronically endorsed by

Marwa Mahmoud Abdelkader Nada

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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